



HealthRate

REIMBURSEMENT MODELING & DECISION SUPPORT

DATA DRIVEN; AI ENABLED REIMBURSEMENT & PAYMENT OPTIMIZATION

January February March April May June July August September October November December



Agenda

01 HealthRate Overview

A brief description of the HealthRate Platform and the HealthRate Team

02 Value Proposition

Our view of the problem and how the HealthRate Platform provides immediate business value

03 Solution Demonstration

Hands on demonstration of the HealthRate Platform.

04 Business Case

Direct discussion on expected benefits and use cases

MEETING WITH YOU TODAY



Management Profiles



Nikki Ahlgren
Chief Commercial Officer

Nikki serves as the Chief Commercial Officer and co-founder of HealthRate and has over 20 years of experience across healthcare leading strategy and growth in the payer and health-tech space.



Jeff Sage
Chief Technology Officer

Jeff serves as the CTO and co-founder of HealthRate and has over 25 years of experience across healthcare with a focus on IT strategy, platform implementation, architecture, software development..



TIM MICHAELS
Chief Executive Officer

Tim is the Founder and CEO of HealthRate and has over 30 years of experience across healthcare with a focus on risk management, analytics, reimbursement strategies, and software development.

OVERVIEW

HealthRate.io



A payment optimization and decision support platform. Built on decades of domain experience, trillions of data points, advanced statistical methods, and recent advancements in AI/ML, HealthRate proactively identifies the optimal source and cost of care.

Quantifiable immediate results. With a focus on transplants, novel therapies, complex oncology, and \$1M+ claims in neonatal, musculoskeletal, and cardiology HealthRate is practical, implementable, and can be easily integrated into your existing processes and technology stack.

The time is now. Transparency legislation has opened Pandora's Box. The healthcare ecosystem continues to face challenges publishing compliant data, accessing MRF data, and applying contract, rate, and claims information in accordance with fiduciary duties. A wave of litigation has put payers and providers alike at great risk

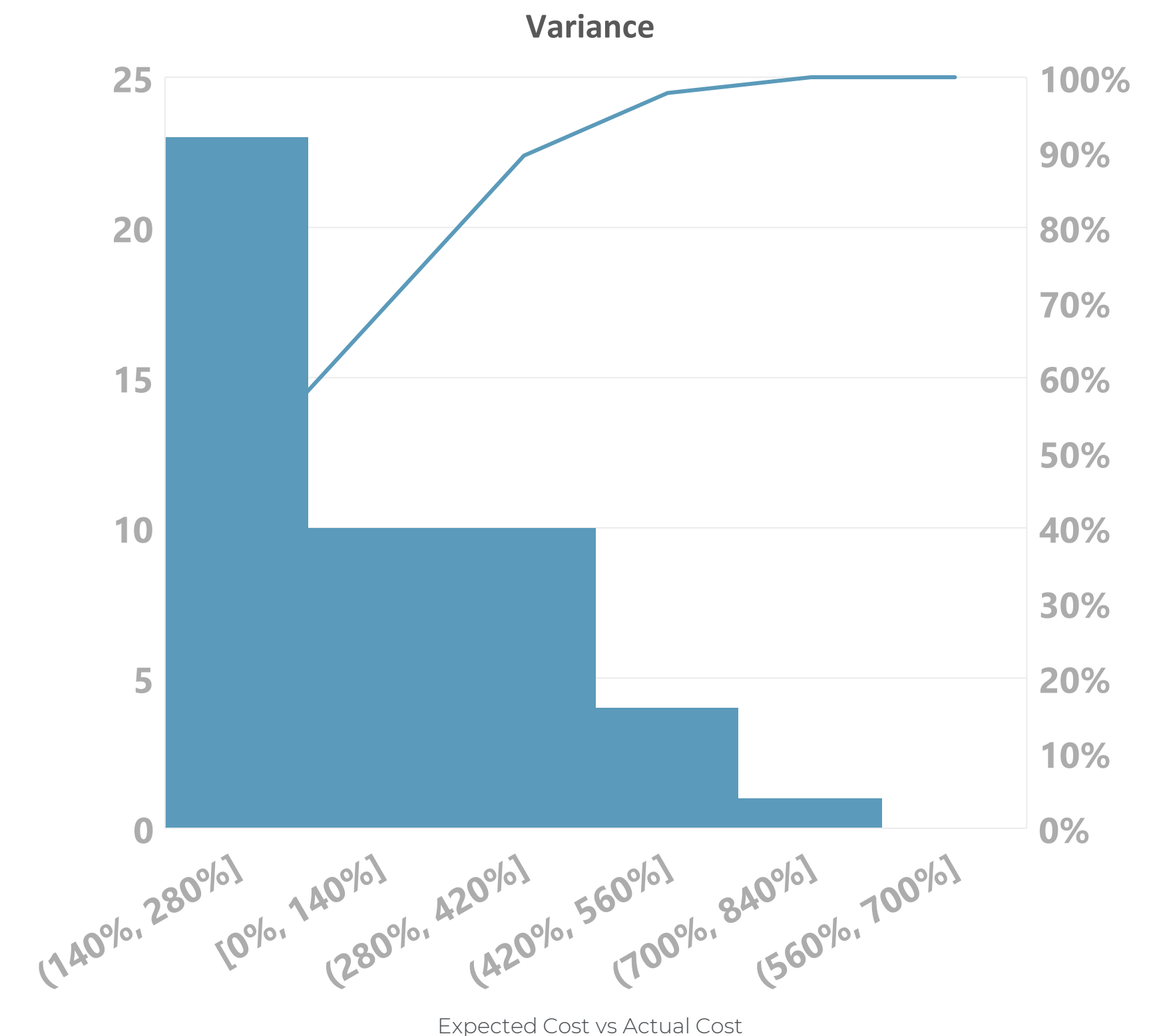
VALUE PROPOSITION - PROBLEM STATEMENT

High dollar healthcare services are procured at highly variable obfuscated rates directly impacting payer bottom line financial performance

A single case can result in \$50,000 - \$3,000,000+ of accretive savings

Provider contracts are notoriously difficult to understand and time consuming to evaluate.

Complex contract terms, uncertain care pathways, and multiple billing scenarios often result in sub optimal performance.



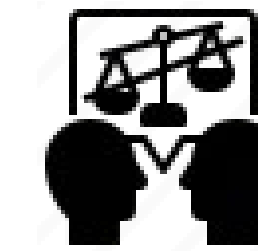
OVERVIEW

Key Tenets



RETURN ON INVESTMENT

Prevents dollars from ever going out the door driving immediate bottom-line impact in the millions



UNBIASED

We maintain complete independence and objectivity and do not accept external compensation



UNPRECEDENTED

No other platform unlocks trillions of rows of transparency data and the power of AI/ML so easily



REAL TIME

HealthRate is always current with your data, available 24x7 and responds within seconds



Username

tim@healthrate.io

Password

Login

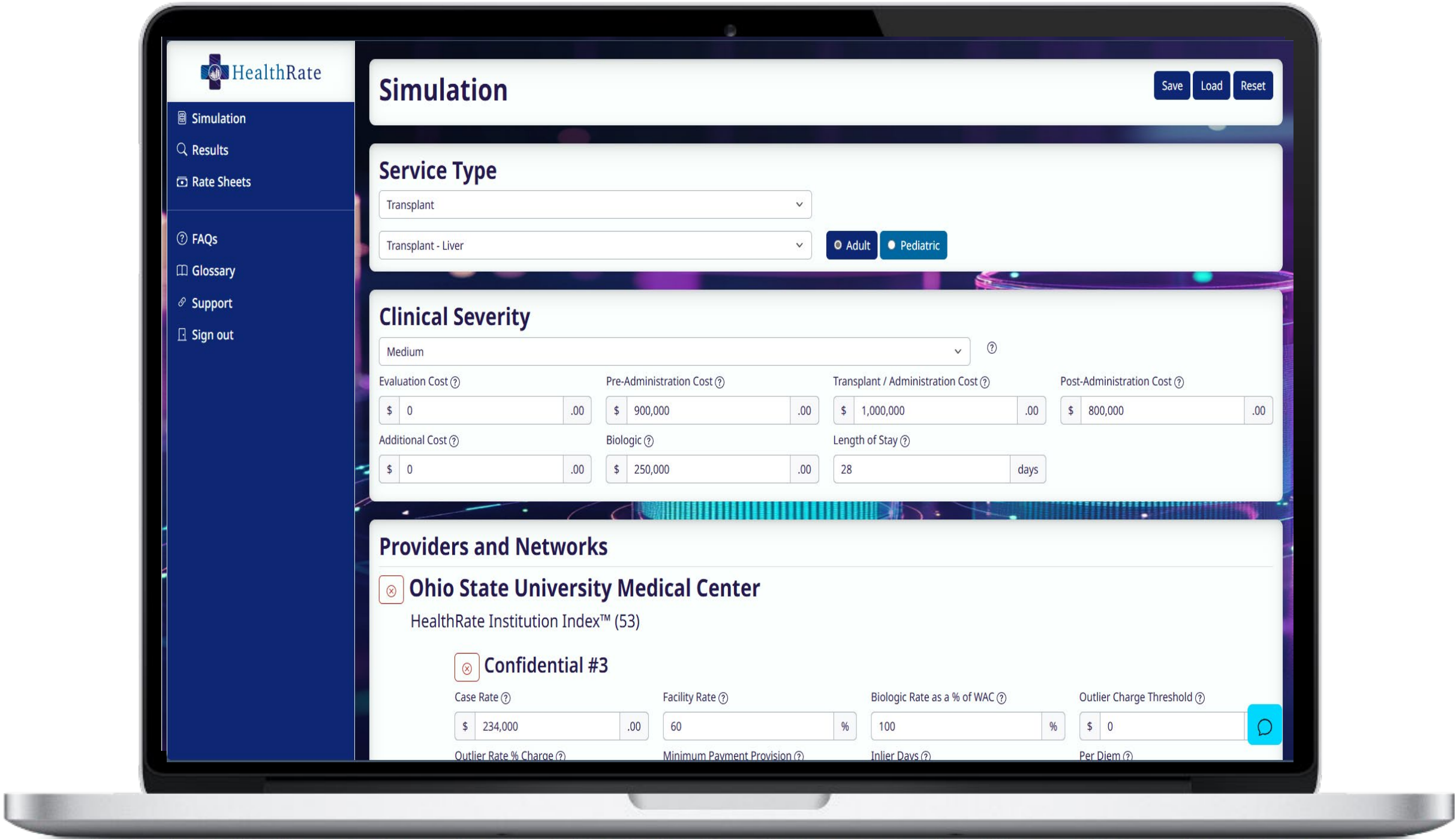
SOLUTION DEMONSTATION

See Clearly

A sophisticated, real-time, decision support platform empowering users to make fact-based decisions every time.

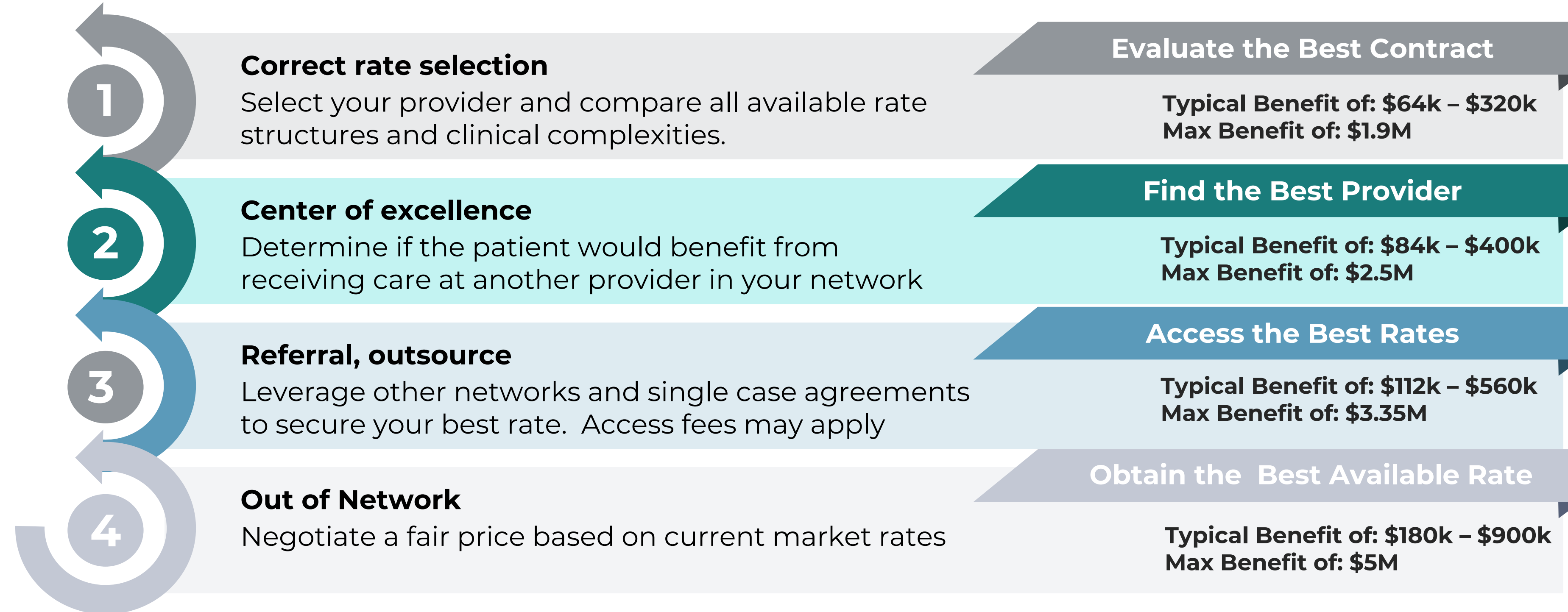
HealthRate quickly and easily puts powerful analysis techniques in the hands of your users with very simple inputs that anyone can use.

Demonstration



Business Use Cases

REAL bottom-line benefits through scenarios that impact your business every day



**Savings are quoted on allowed vs. allowed amounts – NOT billed charges

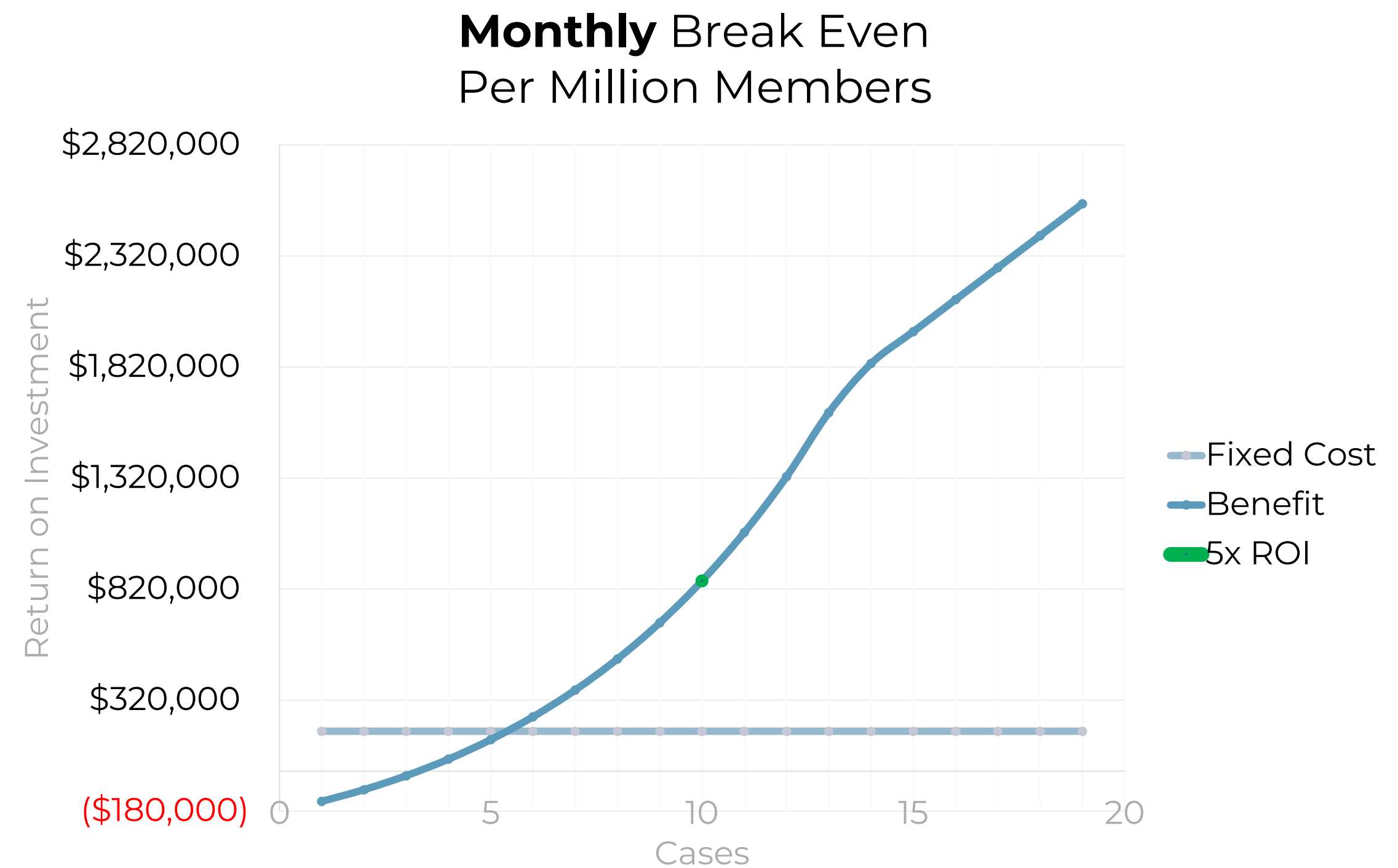
BUSINESS CASE



Return on Investment

Break Even Analysis (Subs. Pricing):

- A typical book of business has a TAM of ~20 cases per one million members with a CAGR of 8% -10%
- A single high complexity case can produce an immediate accretive ROI of 5x or more (above any existing savings)
- Illustration assumes conservative per case benefit ramping to average expected values over time



HealthRate pricing is also available on a shared savings model

VALUE PROPOSITION



HealthRate Process

Know the “Rules”

Rare niche industry experts who have defined the leading industry methodologies, levers, and triggers regarding payment

Which Trillion Records?

AI enabled proprietary database of Transparency Data distilled down to meaningful insights

De-Mystify

Simple yet detailed recommendations with explanations and fully integrated AI contextual intelligence and navigation



Know “What’s Going to Happen”

Carefully constructed scenario database of claims & cost built on the insights from 20+ years of data across 300 million lives

Do the Math!

Implements advanced statistical methodologies and scoring models to enhance accuracy and provide confidence range(s)

Seamless Integration

HealthRate requires minimal training, supports a zero-technology integration option, contains no HIPAA data and was designed with your existing processes in mind



IMMEDIATE BENEFITS INCLUDE:

- No patient data is shared or at risk
- Backend technology integration is supported but optional
- User training can be completed in just a few hours
- Won't disrupt claims or case flow (can use existing prior auth, claims flags, thresholds, etc)
- Integrates seamlessly with your existing process(es) - Simply log into your dedicated www.healthrate.io installation to get started
- Allows your staff to more quickly and accurately assess financial impacts day one
- Enhances staff productivity allowing them to focus more time on care coordination, provider relationships, medical trends, and high value activities

BUSINESS CASE



11

The HealthRate Difference

	HealthRate	Data Providers	Networks	Captive Solution
Sensitive Data @ Risk	X	X	X	X
Platform Learning Curve	X	X	X	X
Turnkey with Your Processes	X	X	X	X
Risk Measurement Capable	X	X	X	X
Advanced Statistical Analysis	X	X	X	X
Immediate ROI	X	X	X	X
Supports Strategy Decisioning	X	X	X	X
Contains Transparency Data	X	X	X	X



HealthRate

The most comprehensive solution available, and the only solution with real-time capability.



Data Providers

Delivers large expensive data sets with no functionality.



Networks

Provide basic if any support tools and are biased towards their product(s).



Captive Solution

Built with limited IT support, typically lack sophisticated analysis capability and rarely support strategic initiatives.

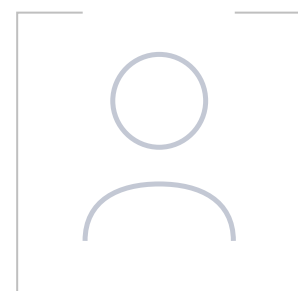
Only HealthRate delivers end-to-end expertise across contracting, clinical, revenue cycle, big data and emerging technology in a unified platform



HealthRate

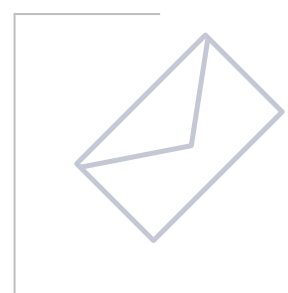
Thank you

FOR YOUR TIME

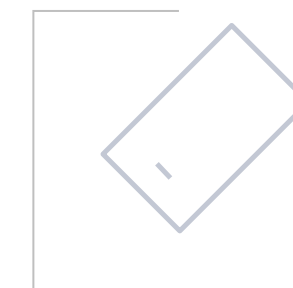


NIKKI AHLGREN

Chief Commercial Officer



NIKKI@HEALTHRATE.IO



612-840-2908



HealthRate

Appendix A

INFOGRAPHICS

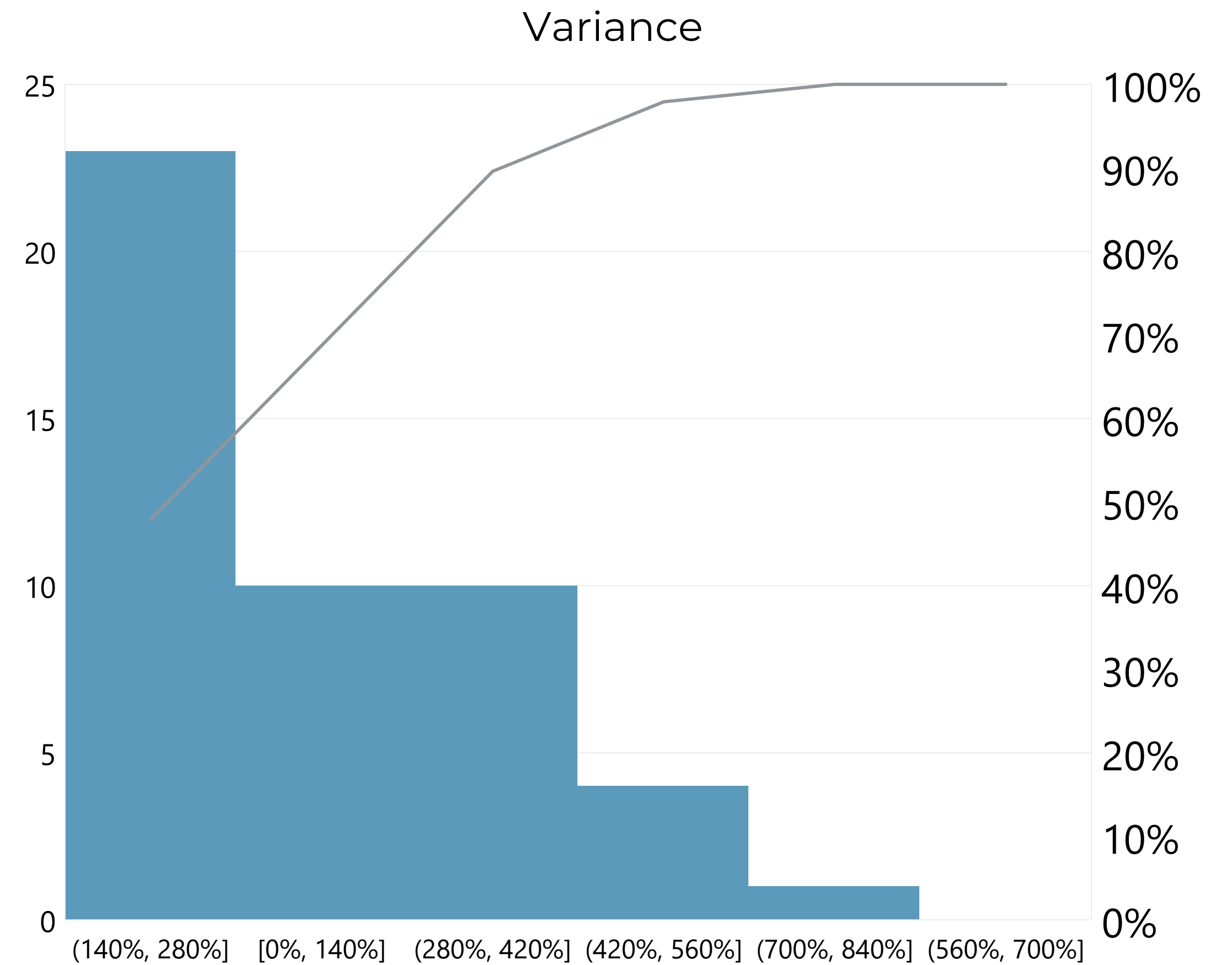
DATA ANALYSIS



Cost Performance

COST VARIANCE:

- Case rate compared to paid amounts
- Data sourced from Network Providers over previous 20 years
- Service types include transplants, cell and gene therapies, and oncology



Over 90% of cases are between 140%-560% of "case rate"

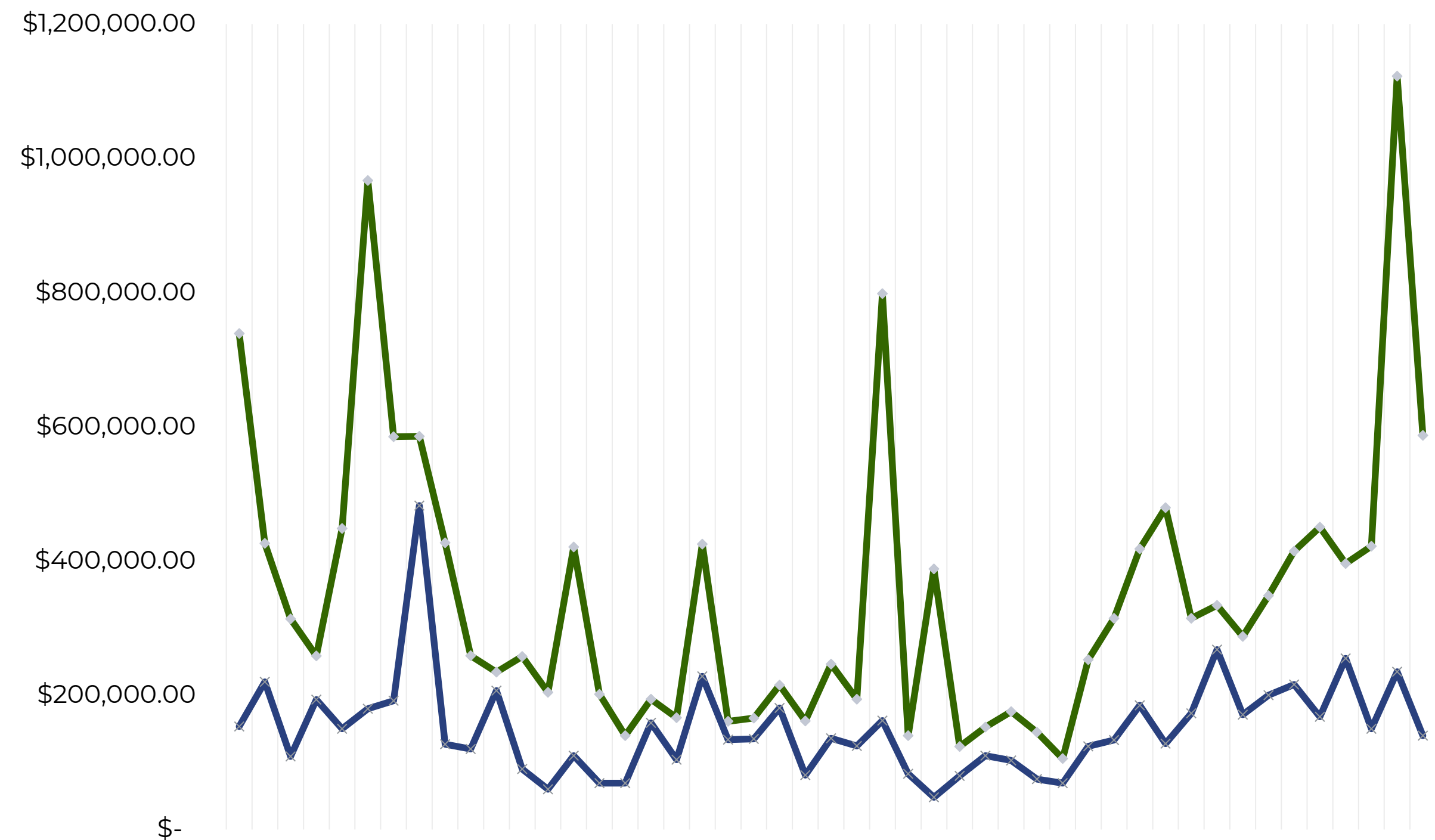
DATA ANALYSIS



Cost Performance

SERVICE TYPE ANALYSIS:

- Case rates vs actual allowed amounts for specific service types and specific rate sets (contracts)
- Examples:
 - Lung Transplants at Duke
 - Auto – BMT at Nebraska



Case rates vs actual ALLOWED amounts



HealthRate

Appendix B

COMPETITIVE SOLUTIONS

CASE STUDIES

MARKET TRENDS

Competitive Solution



CURRENTLY DECISION SUPPORT IS DONE MANUALLY
Primarily by clinical staff or modestly paid administrative staff

- Terms are virtually impossible to understand without the help of modeling and simulation
- Quoted averages are based on a normal distribution when a positively skewed distribution should be utilized
- % of charges do not take into account actual cost of charges (% of what?)
- No ability to consider the severity of the patient
- Calculations in example are incorrect
- The output scenarios are completely invalid
 - The lowest priced option is likely the most expensive in this example

Redacted	Transplant Provider Contract Analysis		
	Type of Transplant	Liver	
	Facility Name	Ohio State University	
	Transplant Network	Network A	Interlink
	Access Fee	\$14,000	\$13,000
	Pre-Transplant Services	65% of BC	60% of BC
	Post Transplant Services		60% of BC
	Marrow/Organ Acquisition	included in case rate	included in case rate
	Transplant Period case rate-facility	\$183,984	\$234,000
	Transplant Period - professional	included in case rate	included in case rate
	Allowable Days		20 inlier days
	Average Length of Stay*	8 days deceased donor/12 days living donor	
	Stoploss threshold (STL)	\$455,988	N/A
	Outlier	CR + 53% of BC above SL threshold	\$10,000 per diem
	Floor Provision/ Minimum Payment Provision (MPP)	NA	50% of BC
	Inliers Provision	Lesser of CR or 75% of BC above outlier	
	Other notes:		
Case Example Calculation using Milliman Average Billed Charges			
	Transplant Network Access Fee	\$14,000	\$13,000
	Pre-transplant = \$46,200	\$30,030	\$27,720
	Procurement = \$104,200		
	Hospital transplant admission = \$490,600	\$288,930.36	\$234,000
	Physician during transplant admission = \$59,200		\$171,000
	Post-transplant = \$140,200	\$91,130	\$84,120
		\$424,090	\$358,840
			\$297,150
		Outlier triggered	

Competitive Solution



ANALYSIS AND DECISION SUPPORT MAY ALSO BE SUPPORTED BY DATA ANALYSTS

- Example contemplates the terms that should be considered when selecting a contract
- Several disclaimers, “N/A”, “see fee schedule”, “only applies”, “each will be different”, etc
- Does not do any calculations, only suggests a framework for variables to consider
- Only relevant for Blues plans and for Cell Therapies – nothing else
- Virtually impossible to understand or utilize

Case Study



REAL WORLD EXAMPLE

- The University of Michigan is a high-volume transplant provider
- By obtaining and decoding University of Michigan data we can see several highly variable service types, for example:
 - Pediatric Heart Transplants
 - Blue Distinctions Centers for Transplant (BDCT) = 38 days - \$225,441
 - Interlink = 40 days - \$194,000, ETS = 40 days - \$181,692
 - Our client can immediately drive a minimum \$43,749 (19%) per case value to any payer utilizing BDCT, by shifting cases to ETS or Interlink

ContractID	Blue Distinctions Centers for Transplant (BDCT)	Emerging Therapy Solutions (previously LifeTrac) / Priority Health	Cigna LifeSource	FrontPath Health Coalition	Interlink	OptumHealth (United Resource Network)	OptumHealth Commun
SOLID ORGAN TRANSPLANTS							
HEART							
Pediatric	38 days - \$225,441	Inlier = 40 days - \$181,692	40 days - \$284,663 / \$676,072 Stop Loss Threshold	\$117,000, 81% for charges >\$187K	40 days - \$194,000	\$235,000 case rate; or never below 60% of Billed Charges	
Adult	38 days - \$171,480	Inlier = 28 days - \$155,736	25 days - \$249,080 / \$676,072 Stop Loss Threshold	\$117,000, 81% for charges >\$187K	28 days - \$166,300	\$235,000; or never below 60% of Billed Charges	
KIDNEY/PANCREAS							
Pediatric	N/A	80% + Organ Procurement paid at OPO Invoice	N/A	\$110,000, 81% for charges >\$176K	N/A	\$125,000; or never below 60% of Billed Charges	
Adult	34 days - \$161,006	Inlier = 11 days - \$107,429	11 days - \$163,681 / \$491,042 Stop Loss Threshold	\$110,000, 81% for charges >\$176K	11 days - \$114,700	\$125,000; or never below 60% of Billed Charges	
PANCREAS							
Pediatric	N/A	80% + Organ Procurement paid at OPO Invoice	N/A	N/A	N/A	\$90,000; or never below 60% of Billed Charges	
Adult	34 days - \$130,971	Inlier = 12 days - \$91,258	11 days - \$128,098 / \$384,294 Stop Loss Threshold	N/A	12 days - \$97,500	\$90,000; or never below 60% of Billed Charges	
KIDNEY-Cadaver Donor							
Pediatric	20 days - \$101,950	Inlier = 11 days - \$77,868	11 days \$99,632 / \$298,896 Stop Loss Threshold	\$65,000, 81% for charges >\$104K	11 days - \$83,200	\$90,000; or never below 60% of Billed Charges	
Adult	20 days - \$95,853	Inlier = 7 days - \$69,216	7 days - \$99,632 / \$298,896 Stop Loss Threshold	\$65,000, 81% for charges >\$104K	7 days - \$73,900	\$90,000; or never below 60% of Billed Charges	
KIDNEY-Live Donor							
Pediatric	20 days - \$101,950	Inlier = 11 days - \$77,868	11 days \$99,632 / \$298,896 Stop Loss Threshold	\$65,000, 81% for charges >\$104K	11 days - \$83,200	\$90,000; or never below 60% of Billed Charges	
Adult	20 days - \$95,853	Inlier = 7 days - \$69,216	7 days - \$99,632 / \$298,896 Stop Loss Threshold	\$65,000, 81% for charges >\$104K	7 days - \$73,900	\$90,000; or never below 60% of Billed Charges	
LIVER-KIDNEY							
Pediatric	N/A	N/A	16 days - \$284,663 / \$676,072 Stop Loss Threshold	N/A	N/A	60% of Billed Charges plus 100% per each organ acquisition	
Adult	39 days - \$264,832 Cadaver	Inlier = 24 days - \$202,292 (Inpatient and Outpatient)	16 days - \$284,663 / \$676,072 Stop Loss Threshold	N/A	N/A	60% of Billed Charges plus 100% per each organ acquisition	
LIVER-Cadaver Donor							
Pediatric	39 days - \$230,790	Inlier = 17 days - \$165,315	17 days - \$249,080 / \$640,491 Stop Loss Threshold	\$150,000, 81% for charges >\$240K	17 days - \$176,600	\$180,000; or never below 60% of Billed Charges	
Adult	39 days - \$218,186	Inlier = 20 days - \$167,684	11 days - \$249,080 / \$640,491 Stop Loss Threshold	\$150,000, 81% for charges >\$240K	20 days - \$179,100	\$180,000; or never below 60% of Billed Charges	
LIVER-Live Donor							
Pediatric	39 days - \$230,790	Inlier = 17 days - \$165,315	17 days - \$249,080 / \$640,491 Stop Loss Threshold	\$150,000, 81% for charges >\$240K	17 days - \$176,600	\$180,000; or never below 60% of Billed Charges	
Adult	39 days - \$225,455 (Notice of Denial)	Inlier = 20 days - \$167,684	20 days - \$249,080 / \$640,491 Stop Loss Threshold	\$150,000, 81% for charges >\$240K	20 days - \$179,100	\$180,000; or never below 60% of Billed Charges	
LUNG							
Pediatric	N/A	80% + Organ Procurement paid at OPO Invoice	N/A	\$135,000, 81% for charges >\$217K	N/A	\$180,000; or never below 60% of Billed Charges	
Adult	38 days - \$174,549	Inlier = 16 days - \$140,492	16 days - \$206,380 / \$619,141 Stop Loss Threshold	\$135,000, 81% for charges >\$217K	16 days - \$150,000	\$180,000; or never below 60% of Billed Charges	
DOUBLE LUNG							
Pediatric	N/A	80% + Organ Procurement paid at OPO Invoice	N/A	N/A	N/A	\$205,000; or never below 60% of Billed Charges	
Adult	38 days - \$225,711	Inlier = 20 days - \$147,548	20 days - \$239,117 / \$676,072 Stop Loss Threshold	N/A	20 days - \$157,600	\$205,000; or never below 60% of Billed Charges	
HEART/LUNG							
Pediatric	N/A	80% + Organ Procurement paid at OPO Invoice	N/A	N/A	N/A	60% of Billed Charges plus 100% per each organ acquisition	

38 days - \$225,441	Inlier = 40 days - \$181,692
38 days - \$171,480	Inlier = 28 days - \$155,736

- We can also run simulations based on actual claims and historical performance of the provider to further optimize financial terms with intelligent use of levers including inlier days, outlier thresholds, floors, etc.

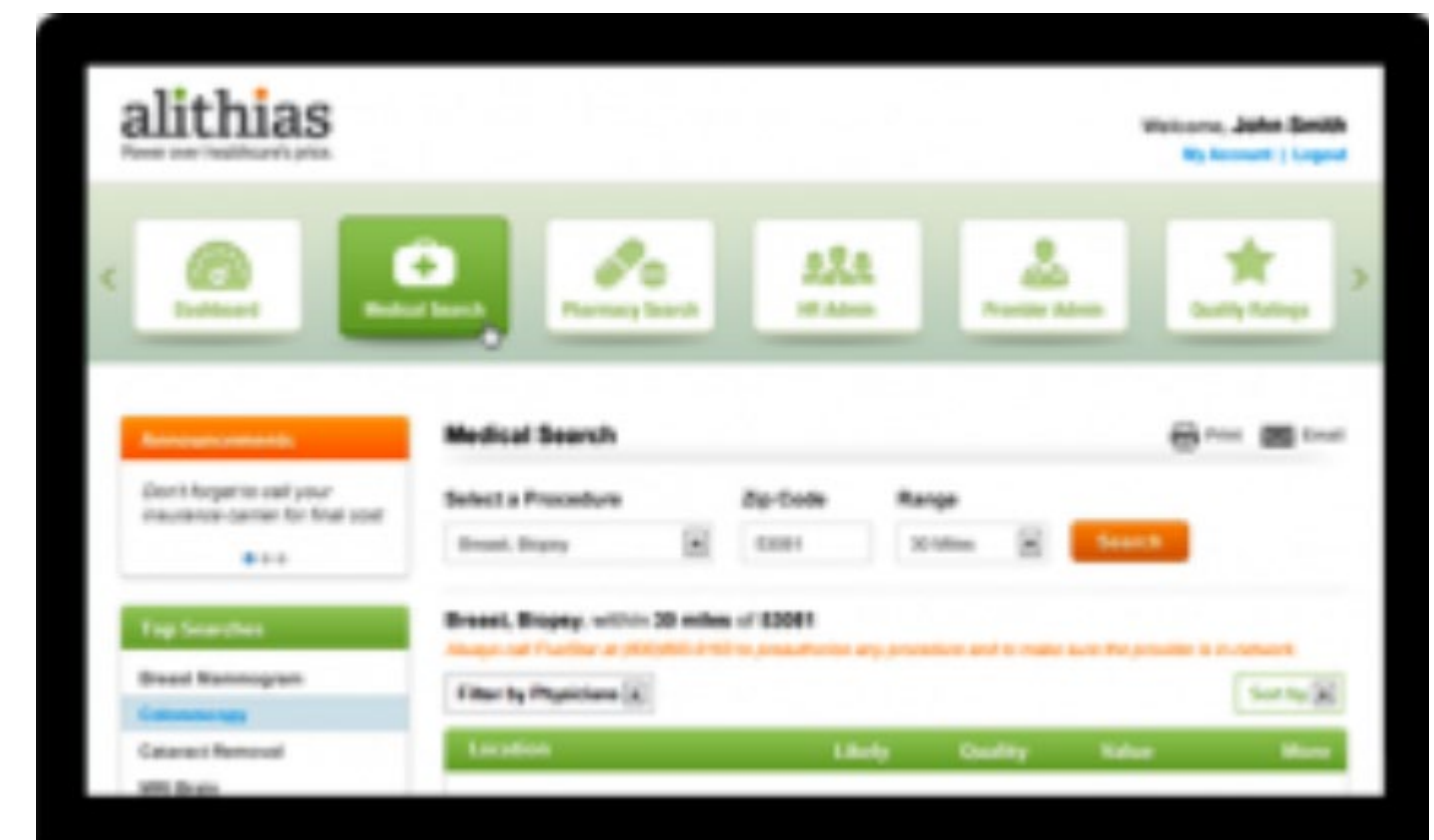
The solution has successfully decoded several transparency files and validated accuracy through multiple data points

CASE STUDY

Client Success Story

"Few employers have access to a solution like HealthRate. Or if they do, the members don't know about it. Incorporating HealthRate into our platform will directly benefit our clients when they need it the most" – Alithias

alithias



PROPRIETARY

Knowledge is power. To overcome an employers' knowledge gap about their data, Alithias uses proprietary algorithms and databases to derive best-in-class cost and quality information, population health risk scoring and the application of predictive modeling scenarios to forecast the financial and health impact of benefits strategies and wellness programs.

EXPENSE REDUCTION

Alithias reduces healthcare expenses for employers and their employees. Savings can then be translated to offer reductions in insurance expenditures or increase benefit plan features. The results are far reaching and provide a rapid and measurable return of the initial investment.

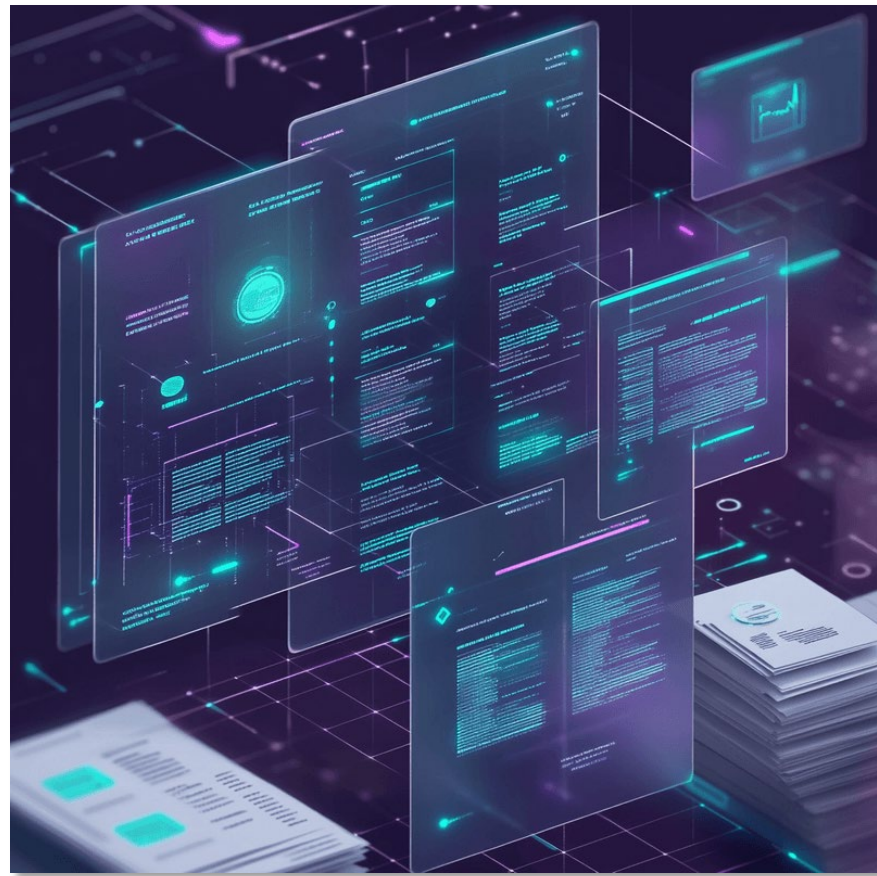
PROACTIVE

That's why proactive, well trained advocates are necessary to educate and coach employees on how to best manage their plan design. Most consumer driven healthcare tools (price transparency websites, mobile apps for benefits information) are utilized at such a low rate because they don't acknowledge that healthcare decisions are reactive. The Alithias program offers multiple points of contact to ensure employees have every opportunity to make good, well-informed decisions.

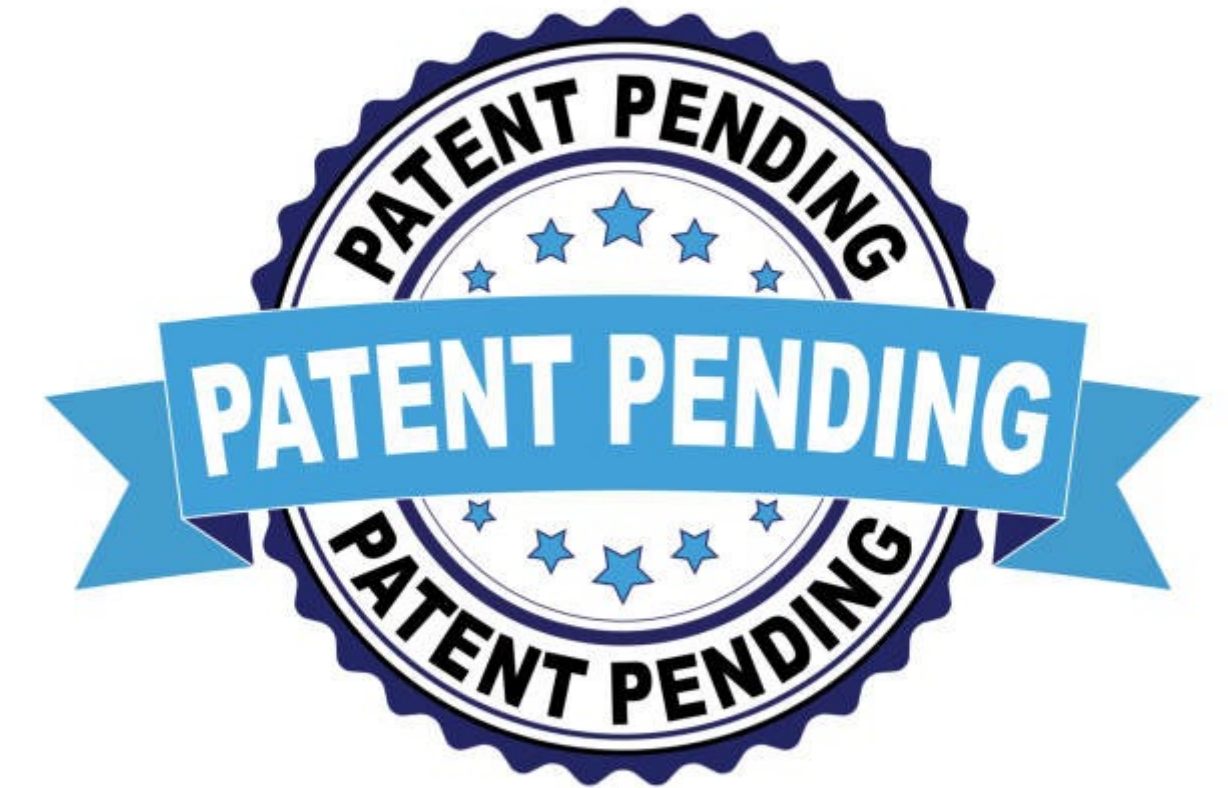
HEALTHRATE INSTITUTIONAL INDEX



HII™



*Tracking and analyzing healthcare costs **across Medical Centers** with precision, the patent pending HealthRate Institutional Index informs smarter financial decisions through cost normalization.*



Provisional patent # 63/707,441

WHAT IS THE HII™

The HealthRate Institutional Index is a patent pending price index that has been developed by normalizing a common basket of relevant services across medical centers. The index allows purchasers to compare costs regardless of charge master disclosure methodologies.

HOW IS HII™ CALCULATED

The HealthRate Institutional Index begins by identifying a comprehensive set of services that are relevant to high-cost cases—a challenging endeavor. It then gathers the necessary data and organizes this information into categories weighted by their significance to service line billing patterns. The index calculates price fluctuations for each category, quantifying these changes according to their weighted importance. Finally, the index is regularly updated to incorporate newly available data and reflect ongoing trends.

THE BENEFIT OF HII™

Price indexing is a well-established and reliable method for comparing costs over time and across different regions. It has been widely used by governments, economists, businesses, and financial institutions for many decades. The HII has been specifically designed to solve the “percent of what” problem; allowing you to compare complex reimbursement methodologies across medical centers with confidence.

MARKET TRENDS

Liability?

Employers are likely to face legal challenges by employees for paying more than is necessary after being warned for years that previously hidden information on negotiated prices for health care services has become publicly available to potential plaintiffs through new laws and regulations. These new lawsuits would be comparable in some ways to those filed against companies for paying excessive fees or mismanaging investments in 401(k) defined contribution retirement plans, according to attorneys who advise employers.

- Employers can mitigate their liability exposure to these types of cases by ensuring they are engaged in a prudent process in selecting and monitoring their service providers
- It is no longer a sufficient defense for employers to rely on brokers and plan administrators “to do the right thing”
- Employers may need to add staff, spend more time examining contracts to ensure they’re getting the best deal, “and then do a lot of digging to ensure that the people that they’ve signed contracts with are actually doing what they say they will do,”

“Companies have taken the unusual step in recent years of suing their health plan administrators for breaching fiduciary duties”

<https://news.bloomberglaw.com/daily-labor-report/johnson-johnson-case-signals-employee-drug-price-suits-to-come>

1/7/2002

1/8/2002

1/9/2002